DIAGNOSTIC REPORT

MC-6469

agilus diagnostics

PATIENT NAME : MRS. ARTI KUMAR **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000096053
JAY SHREE PATIENT CARE CENTRE
SHOP NO.24, SHAMINA MARKET, OPPOSITE K.G.M.C., NEW OPD BUILDING CHOWK,
LUCKNOW 226003
9956588890 8090988890

ACCESSION NO : **0024YJ000127**
PATIENT ID : ARTIF895459770
CLNT.PATIENT ID:
ABHA NO :

AGE/SEX :43 Years Female
DRAWN :01/10/2025 11:23:42
RECEIVED : 01/10/2025 12:52:44
REPORTED :01/10/2025 16:33:42

Test Report Status **Preliminary**	Results	Biological Reference Interval	Units

HAEMATOLOGY - CBC

COMPLETE CARE ACTIVE PRO WOMEN

BLOOD COUNTS, EDTA WHOLE BLOOD

HEMOGLOBIN (HB) METHOD : SPECTROPHOTOMETRY	**11.4 Low**	12.0 - 15.0	g/dL
RED BLOOD CELL (RBC) COUNT METHOD : ELECTRICAL IMPEDANCE	4.29	3.8 - 4.8	mil/µL
WHITE BLOOD CELL (WBC) COUNT METHOD : ELECTRICAL IMPEDANCE	6.33	4 - 10	thou/µL
PLATELET COUNT METHOD : ELECTRICAL IMPEDANCE/MICROSCOPY	396	150 - 410	thou/µL

RBC AND PLATELET INDICES

HEMATOCRIT (PCV) METHOD : ELECTRONIC IMPEDANCE/CALCULATION	36.6	36.0 - 46.0	%
MEAN CORPUSCULAR VOLUME (MCV) METHOD : CALCULATED PARAMETER	85.3	83.0 - 101.0	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH) METHOD : CALCULATED PARAMETER	**26.6 Low**	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION(MCHC) METHOD : CALCULATED PARAMETER	**31.2 Low**	31.5 - 34.5	g/dL
RED CELL DISTRIBUTION WIDTH (RDW) METHOD : CALCULATED PARAMETER	**14.5 High**	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV) METHOD : CALCULATED PARAMETER	8.4	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT

NEUTROPHILS METHOD : MICROSCOPY /IMPEDANCE	63	40 - 80	%
LYMPHOCYTES METHOD : MICROSCOPY /IMPEDANCE	30	20 - 40	%
MONOCYTES METHOD : MICROSCOPY /IMPEDANCE	04	2.0 - 10.0	%

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ULR No.775000014538119-0024

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EOSINOPHILS METHOD : MICROSCOPY /IMPEDANCE	03	1 - 6	%
BASOPHILS METHOD : MICROSCOPY /IMPEDANCE	00	0 - 2	%
ABSOLUTE NEUTROPHIL COUNT METHOD : CALCULATED PARAMETER	3.99	2 - 7	thou/µL
ABSOLUTE LYMPHOCYTE COUNT METHOD : CALCULATED PARAMETER	1.90	1.0 - 3.0	thou/µL
ABSOLUTE MONOCYTE COUNT METHOD : CALCULATED PARAMETER	0.25	0.20 - 1.00	thou/µL
ABSOLUTE EOSINOPHIL COUNT METHOD : CALCULATED PARAMETER	0.19	0.02 - 0.50	thou/µL

Interpretation(s)
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease. (Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.

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HAEMATOLOGY

COMPLETE CARE ACTIVE PRO WOMEN

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD

HBA1C	5.5	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 ADA Target: 7.0 Action suggested: > 8.0	%
METHOD : HPLC			
ESTIMATED AVERAGE GLUCOSE(EAG)	111.2	< 116	mg/dL
METHOD : CALCULATED PARAMETER			

Interpretation(s)
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-**Used For**:

1. Evaluating the long-term control of blood glucose concentrations in diabetic patients.
2. Diagnosing diabetes.
3. Identifying patients at increased risk for diabetes (prediabetes).
The ADA recommends measurement of HbA1c (typically 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to determine whether a patients metabolic control has remained continuously within the target range.
1. eAG (Estimated average glucose) converts percentage HbA1c to md/dl, to compare blood glucose levels.
2. eAG gives an evaluation of blood glucose levels for the last couple of months.
3. eAG is calculated as eAG (mg/dl) = 28.7 * HbA1c - 46.7

HbA1c Estimation can get affected due to :
1. Shortened Erythrocyte survival : Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss,hemolytic anemia) will falsely lower HbA1c test results.Fructosamine is recommended in these patients which indicates diabetes control over 15 days.
2.Vitamin C & E are reported to falsely lower test results.(possibly by inhibiting glycation of hemoglobin.
3. Iron deficiency anemia is reported to increase test results. Hypertriglyceridemia,uremia, hyperbilirubinemia, chronic alcoholism,chronic ingestion of salicylates & opiates addiction are reported to interfere with some assay methods,falsely increasing results.
4. Interference of hemoglobinopathies in HbA1c estimation is seen in

a) Homozygous hemoglobinopathy. Fructosamine is recommended for testing of HbA1c.
b) Heterozygous state detected (D10 is corrected for HbS & HbC trait.)
c) HbF > 25% on alternate paltform (Boronate affinity chromatography) is recommended for testing of HbA1c.Abnormal Hemoglobin electrophoresis (HPLC method) is recommended for detecting a hemoglobinopathy

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BIOCHEMISTRY

COMPLETE CARE ACTIVE PRO WOMEN

GLUCOSE FASTING,FLUORIDE PLASMA

FBS (FASTING BLOOD SUGAR) METHOD : HEXOKINASE	**118 High**	(Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024)	mg/dL

KIDNEY FUNCTION TEST, SERUM

BLOOD UREA NITROGEN METHOD : UREASE - UV	8	6 - 20	mg/dL
CREATININE METHOD : ALKALINE PICRATE-KINETIC	0.60	0.60 - 1.10	mg/dL
BUN/CREAT RATIO METHOD : CALCULATED PARAMETER	13.33	5.00 - 15.00	
URIC ACID METHOD : URICASE, COLORIMETRIC	**7.0 High**	2.4 - 5.7	mg/dL
TOTAL PROTEIN METHOD : BIURET,SERUM BLANK,ENDPOINT	7.1	6.6 - 8.7	g/dL
ALBUMIN METHOD : BROMOCRESOL GREEN	4.3	3.97 - 4.94	g/dL
GLOBULIN METHOD : CALCULATED PARAMETER	2.8	2.0 - 4.0 Neonates - Pre Mature: 0.29 - 1.04	g/dL
CALCIUM METHOD : BAPTA	9.7	8.6 - 10.0	mg/dL
SODIUM, SERUM METHOD : ISE INDIRECT	140	135 - 145	mmol/L
POTASSIUM, SERUM METHOD : ISE INDIRECT	4.49	3.5 - 5.3	mmol/L
CHLORIDE, SERUM METHOD : ISE INDIRECT	104	97 - 110	mmol/L

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LIVER FUNCTION PROFILE, SERUM

BILIRUBIN, TOTAL METHOD : DIAZONIUM ION, BLANKED (ROCHE)	0.84	Upto 1.2	mg/dL
BILIRUBIN, DIRECT METHOD : DIAZOTIZATION	**0.31 High**	< or = 0.30	mg/dL
BILIRUBIN, INDIRECT METHOD : CALCULATED PARAMETER	0.53	0.00 - 0.60	mg/dL
TOTAL PROTEIN METHOD : BIURET,SERUM BLANK,ENDPOINT	7.1	6.6 - 8.7	g/dL
ALBUMIN METHOD : BROMOCRESOL GREEN	4.3	3.97 - 4.94	g/dL
GLOBULIN METHOD : CALCULATED PARAMETER	2.8	2.0 - 4.0 Neonates - Pre Mature: 0.29 - 1.04	g/dL
ALBUMIN/GLOBULIN RATIO METHOD : CALCULATED PARAMETER	1.5	1.0 - 2.0	RATIO
ASPARTATE AMINOTRANSFERASE(AST/SGOT) METHOD : UV WITHOUT P5P	22	0 - 32	U/L
ALANINE AMINOTRANSFERASE (ALT/SGPT) METHOD : UV WITHOUT P5P	31	0 - 31	U/L
ALKALINE PHOSPHATASE METHOD : PNPP, AMP BUFFER-IFCC	**106 High**	35 - 105	U/L
GAMMA GLUTAMYL TRANSFERASE (GGT) METHOD : G-GLUTAMYL-CARBOXY-NITROANILIDE-IFCC	**40 High**	5 - 36	U/L
LACTATE DEHYDROGENASE METHOD : L TO P, IFCC	159	135 - 214	U/L

PHOSPHORUS, SERUM

PHOSPHORUS METHOD : PHOSPHOMOLYBDATE METHOD	4.1	2.5 - 4.5	mg/dL

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MAGNESIUM, SERUM			
MAGNESIUM, SERUM	1.8	1.6 - 2.6	mg/dL
METHOD : XYLIDYL BLUE			

Interpretation(s)
GLUCOSE FASTING,FLUORIDE PLASMA-**TEST DESCRIPTION**
Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.
Increased in:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.
Decreased in :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease, malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol sulfonylureas,tolbutamide,and other oral hypoglycemic agents.
NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.
High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.
LIVER FUNCTION PROFILE, SERUM-
Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice.**Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors &Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.
AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver,liver cancer,kidney failure,hemolytic anemia,pancreatitis,hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles, and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.AST levels increase during acute hepatitis,sometimes due to a viral infection,ischemia to the liver,chronic hepatitis,obstruction of bile ducts,cirrhosis.
ALP is a protein found in almost all body tissues.Tissues with higher amounts of ALP include the liver,bile ducts and bone.Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease,Rickets,Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia,Malnutrition,Protein deficiency,Wilsons disease.
GGT is an enzyme found in cell membranes of many tissues mainly in the liver,kidney and pancreas.It is also found in other tissues including intestine,spleen,heart, brain and seminal vesicles.The highest concentration is in the kidney,but the liver is considered the source of normal enzyme activity.Serum GGT has been widely used as an index of liver dysfunction.Elevated serum GGT activity can be found in diseases of the liver,biliary system and pancreas.Conditions that increase serum GGT are obstructive liver disease,high alcohol consumption and use of enzyme-inducing drugs etc.
Total Protein also known as total protein,is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.Higher-than-normal levels may be due to:Chronic inflammation or infection,including HIV and hepatitis B or C,Multiple myeloma,Waldenstroms disease.Lower-than-normal levels may be due to: Agammaglobulinemia,Bleeding (hemorrhage),Burns,Glomerulonephritis,Liver disease, Malabsorption,Malnutrition,Nephrotic syndrome,Protein-losing enteropathy etc.
Albumin is the most abundant protein in human blood plasma.It is produced in the liver.Albumin constitutes about half of the blood serum protein.Low blood albumin levels (hypoalbuminemia) can be caused by:Liver disease like cirrhosis of the liver, nephrotic syndrome,protein-losing enteropathy,Burns,hemodilution,increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc
MAGNESIUM, SERUM-Description- Magnesium is primariliy an intracellular ion associated with GI absorption and renal excretion. Second most abundant ion in bone.It functions as co-factor in numerous enzymes e.g. ATPase . 65-70% of Mg is in ionized state and nearly 35% is protein bound.

Interpretation-
Increased in- Dehydration, Tissue trauma, Renal failure, Hypothyroidism, excessive intake of antacid.
Decrease in- Chronic diarrhea, Enteric fistula, Starvation, Chronic alcoholism, Total parenteral Nutrition, Diuretics.

Note- Hypomagnesemia is associated with weakness, tetany, disorientation and somnolence

Limitation-
• Hemolysis yields elevated levels of Mg being an intracellular ion.

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• Serum magnesium levels may remain normal even when total body stores of magnesium are depleted upto 20%

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BIOCHEMISTRY - LIPID

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LIPID PROFILE WITH CALCULATED LDL,SERUM

CHOLESTEROL, TOTAL	**204 High**	< 200 Desirable 200 - 239 Borderline High >/= 240 High	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE,PEROXIDASE			
TRIGLYCERIDES	**249 High**	< 150 Normal 150 - 199 Borderline High 200 - 499 High >/= 500 Very High	mg/dL
METHOD : ENZYMATIC, END POINT			
HDL CHOLESTEROL	40	< 40 Low >/=60 High	mg/dL
METHOD : DIRECT MEASURE POLYMER-POLYANION			
CHOLESTEROL LDL	**114 High**	< 100 Optimal 100 - 129 Near or above optimal 130 - 159 Borderline High 160 - 189 High >/= 190 Very High	mg/dL
NON HDL CHOLESTEROL	**164 High**	Desirable: Less than 130 Above Desirable: 130 - 159 Borderline High: 160 - 189 High: 190 - 219 Very high: > or = 220	mg/dL
METHOD : CALCULATED PARAMETER			
VERY LOW DENSITY LIPOPROTEIN	**49.8 High**	Desirable value : 10 - 35	mg/dL
CHOL/HDL RATIO	**5.1 High**	3.3-4.4 Low Risk 4.5-7.0 Average Risk 7.1-11.0 Moderate Risk > 11.0 High Risk	
LDL/HDL RATIO	2.8	0.5 - 3.0 Desirable/Low Risk 3.1 - 6.0 Borderline/Moderate Risk >6.0 High Risk	

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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors

Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Interpretation of Diagnostic Tests by Jacques Wallach, 11th Ed.

Friedewald, W. T., Levy, R. I., & Fredrickson, D. S. (1972). "Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge."Clinical Chemistry, 18(6), 499-502.

National Cholesterol Education Program (NCEP) Adult Treatment Panel III (ATP III). (2001).

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Third Report of the National Cholesterol Education Program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III) Final Report. Circulation, 106(25), 3143-3421.

American College of Cardiology (ACC) and American Heart Association (AHA) (2013). 2013 ACC/AHA Guideline on the Assessment of Cardiovascular Risk. Circulation, 129(25), S49-S73.

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NEPHELOMETRY

COMPLETE CARE ACTIVE PRO WOMEN

HSCRP (HIGH SENSITIVITY C-REACTIVE PROTEIN),SERUM			
HIGH SENSITIVITY CRP	**13.87 High**	Relative risk for CVD: < 1.0 Low Risk 1.0 - 3.0 Average Risk > 3.0 High Risk	mg/L
METHOD : PARTICLE ENHANCED IMMUNOTURBIDIMETRIC ASSAY			
RF FACTOR (RHEUMATOID FACTOR QUANTITATIVE),SERUM			
RHEUMATOID FACTOR	<10.0	< 14.0	IU/mL
METHOD : IMMUNOTURBIDIMETRIC ASSAY			

Interpretation(s)
HSCRP (HIGH SENSITIVITY C-REACTIVE PROTEIN),SERUM-High sensitivity CRP measurements may be used as an independent risk marker for the identification of individuals at risk for future cardiovascular disease. Measurement of hs- CRP, when used in conjunction with traditional clinical laboratory evaluation of acute coronary syndromes, may be useful as an independent marker of prognosis for recurrent events, in patients with stable coronary disease or acute coronary syndromes.
When using this assay for risk assessment, patients with persistently unexplained, marked elevation of hs- CRP (> 10mg/l) after repeated testing should be evaluated for non cardiovascular etiologies. In Rheumatic and other inflammatory diseases, value of CRP less than 10 mg/l is considered satisfactory. More than 10 mg/l suggests disease activity. Patients with evidence of active infection, systemic inflammatory processes or trauma should not be tested for cardiovascular disease risk assessment until these conditions have abated
Hs- CRP levels should not be substituted for assessment of traditional cardiovascular risk factors.
Turbidity and particles in the sample may interfere with the determination. Patient samples which contain heterophilic antibodies could react in immunoassays to give a falsely elevated or depressed result.
Results of this test should always be interpreted in conjunction with the patient's medical history, clinical presentation and other findings.

References:

1. Teitz textbook of clinical chemistry and Molecular diagnostics, edited by Carl A Burtis, Edward R. Ashrwood, David E Bruns, 4th edition, Elseiver publication, 2006,962-966
2. Parson TA, Mensah GA, et al. Marker of inflammation and cardiovascular disease: application to clinical and public health practice. Circulation 2003,107,499-511
3. Rheumatoid arthritis disease activity measures: American College of Rheumatology recommendations for use in clinical practice: Jacyln Anderson, Liron Caplin et al, Wiley online, 2012.
RF FACTOR (RHEUMATOID FACTOR QUANTITATIVE),SERUM-This test is used for diagnosis of Rheumatoid arthritis (RA) in individuals with a suggestive clinical presentation.

Rheumatoid factor is an IgM autoantibody directed against the Fc portion of Immunoglobulin G (IgG) and is found in more than two-thirds of adults with Rheumatoid arthritis. Detection of RF is one of the criteria of the American Rheumatology Association (ARA) for the diagnosis of Rheumatoid arthritis.

The presence of Rheumatoid factor is of prognostic significance also, since patients with high titres tend to have more severe and progressive disease.

RF is also found in a number of other conditions such as Systemic lupus erythematosus, Sjogren's syndrome, chronic liver disease, hepatitis B. It plays an important role in differential diagnosis between RA and other rheumatic diseases.

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ENDOCRINOLOGY

COMPLETE CARE ACTIVE PRO WOMEN

THYROID PANEL II (FT3,FT4,TSH) SERUM

FREE TRIIODOTHYRONINE (FT3)	3.56	2.0 - 4.4	pg/mL
FREE THYROXINE (FT4)	1.38	0.93 - 1.70	ng/dL

METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY

TSH (ULTRASENSITIVE)	3.960	Non Pregnant Women 0.27 - 4.20 Pregnant Women (As per American Thyroid Association) 1st Trimester 0.100 - 2.500 2nd Trimester 0.200 - 3.000 3rd Trimester 0.300 - 3.000	µIU/mL

METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY

Interpretation(s)

Sr. No.	TSH	FT4	FT3	Possible Conditions
1	High	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	(1)Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3)Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	(1) Subclinical Hyperthyroidism
6	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor

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7	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism
8	Normal/Low	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association duriing pregnancy and Postpartum, 2011.

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SPECIALISED CHEMISTRY - ANEMIA

COMPLETE CARE ACTIVE PRO WOMEN

IRON AND TIBC STUDIES,SERUM

IRON	65	37 - 145	µg/dL
METHOD : FEROZINE - NO DEPROTEINIZATION			
TOTAL IRON BINDING CAPACITY	375	250 - 450	µg/dL
METHOD : CALCULATED PARAMETER			
% SATURATION	17	13 - 45	%

Interpretation(s)
IRON AND TIBC STUDIES,SERUM-Total iron binding capacity (TIBC) measures the blood's capacity to bind iron with transferrin and thus is an indirect way of assessing transferrin level.
Taken together with serum iron and percent transferrin saturation this test is performed when they is a concern about anemia, iron deficiency or iron deficiency anemia. However, because the liver produces transferrin, alterations in liver function (such as cirrhosis, hepatitis, or liver failure) must be considered when performing this test.
Increased in:
- iron deficiency
- acute and chronic blood loss
- acute liver damage
- progesterone birth control pills

Decreased in:
- hemochromatosis
- cirrhosis of the liver
- thalassemia
- anemias of infection and chronic diseases
- nephrosis
- hyperthyroidism

The percent Transferrin saturation = Serum Iron/TIBC x 100
Unsaturated Binding Capacity (UIBC)=TIBC - Serum Iron.
Limitations: Estrogens and oral contraceptives increase TIBC and Asparaginase, chloramphenicol, corticotropin, cortisone and testosterone decrease the TIBC level.

Reference:
1.Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R.Ashwood, David E Bruns, 4th Edition, Elsevier publication, 2006, 563, 1314-1315.
2. Wallach's Interpretation of Diagnostic tests, 9th Edition, Ed Mary A Williamson and L Michael Snyder. Pub Lippincott Williams and Wilkins, 2011, 234-235.

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SPECIALISED CHEMISTRY - HORMONE

COMPLETE CARE ACTIVE PRO WOMEN	RESULT PENDING
CORTISOL,SERUM	RESULT PENDING

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SPECIALISED CHEMISTRY - TUMOR MARKER

COMPLETE CARE ACTIVE PRO WOMEN

CA 125 (OVARIAN CANCER MONITOR),SERUM

CA 125	15.0	0.00 - 35.00	U/mL

Interpretation(s)

CA 125 (OVARIAN CANCER MONITOR),SERUM-CA 125 is a surface antigen, identified as a 200-1000 kDa mucin-like glycoprotein associated with non-mucinous epithelial ovarian malignancy. CA 125 is a useful tumor marker for evaluating therapy and monitoring disease status in patients under treatment for ovarian cancer. Measured serially the levels of CA 125 correspond with disease progression or regression. The rate of change in CA 125 is also highly prognostic. As a diagnostic tool however, the level of CA 125 alone is not sufficient to determine the presence or extent of disease. Levels of CA 125 should not be interpreted as absolute evidence of the presence or the absence of malignant diseases. Before treatment, patients with confirmed ovarian carcinoma frequently have levels of CA 125 within the range observed in healthy individuals. Preoperative levels of CA 125 in patients with malignant pelvic masses provide no information regarding the histological grade or diameter of the tumor mass.

Elevated levels of CA 125 can be observed in patients with nonmalignant diseases. Patients with certain benign conditions, such as hepatic cirrhosis, acute pancreatitis, endometriosis, pelvic inflammatory disease, menstruation and first trimester pregnancy show elevated levels of CA 125. Elevated levels are also found in 1 to 2% of healthy donors.

Measurements of CA 125 should always be used in conjunction with other diagnostic procedures, including information from the patients clinical evaluation. The concentration of CA 125 in a given specimen determined with assays from different manufacturers can vary due to differences in assay methods, calibration, and reagent specificity. Values obtained with different assay methods cannot be used interchangeably. Heterophilic antibodies in human serum can react with reagent immunoglobulins, interfering with in vitro immunoassays. Patients routinely exposed to animals or to animal serum products can be prone to this interference and anomalous values may be observed

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SPECIALISED CHEMISTRY - VITAMIN

COMPLETE CARE ACTIVE PRO WOMEN

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM

25 - HYDROXYVITAMIN D	40.09	Deficiency: < 20.0 Insufficiency: 20.0 - < 30.0 Sufficiency: 30.0 -100.0 Toxicity > 100.0	ng/mL

VITAMIN B12(CYANOCOBALAMINE), SERUM

VITAMIN B12	390.3	Normal adults: 197-771	pg/mL

FOLIC ACID, SERUM

FOLIC ACID	12.60	3.89 - 26.80	ng/mL

Interpretation(s)
25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM- **Test description**
Vitamin D has anti-inflammatory and immune-modulating properties and it works towards the bones, teeth, intestines, immune system, pancreas, muscles and brain. It helps to maintain normal calcium and phosphate levels.Vitamin D is a fat-soluble vitamin. Also called as "Sunshine Vitamin".Two main forms as Cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight (Type B UV) exposure & Ergocalciferol (vitamin D2) present mainly in dietary sources.
Vit D25(OH)D deficiency is seen due to poor or inadequate sunlight exposure, Nutritional or dietary deficiency or fat malabsorption, Severe Hepatocellular disease, Secondary hyperparathyroidism, Hypocalcemia tetany which can cause involuntary contraction of muscles, leading to cramps and spasms, Rickets in children, Osteomalacia in adults- due to vitamin D deficiency mainly, Older adults- osteoporosis. (Increased risk of bone fractures)due to long-term effect of calcium and/or vitamin D deficiency, Other conditions that are precipitated by Vit D deficiency included increased cardiovascular risk, low immunity & chronic renal failure.
Elevated levels may be seen in patients taking supplements(hence recommended to repeat after 3 months for estimation of accurate levels), Vitamin D intoxication, sarcoidosis and malignancies containing non regulated 1-alpha hydroxylase in the lesion.
Recommendations
1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample 2.25(OH)D is the analyte of choice for determination of the Vitamin D status as it is the major storage & active form of Vitamin D and has longer half-life. 3. Kidney Disease Outcomes Quality Initiatives (KDOQI) and Kidney Disease Improving Global Outcomes (KDIGO) recommend activated vitamin D testing for CKD patients.
Note-Our Vitamin D assays is standardized to be in alignment with the ID-LC/MS/MS 25(OH)vitamin D Reference Method Procedure (RMP), the reference procedure for the Vitamin D Standardization Program (VDSP). The VDSP, a collaboration of the National Institutes of Health Office of Dietary Supplements, National Institute of Technology and Standards, Centers for Disease Control and Ghent University, is an initiative to standardize 25(OH)vitamin D measurement across methods.
Reference: 1.Wallach Interpretation of diagnostic test, 10th edition.
VITAMIN B12(CYANOCOBALAMINE), SERUM-**Test description**
1.Measures the amount of Vitamin B12/ Cyanocobalamin or Methyl cobalamin in blood.2. Done in Anemic conditions like Megaloblastic anemia, pernicious anemia, dietary

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folate deficiencies,3.Workup of neuropathies especially due to diabetes.4.Nerve health and it is monitored in treatment of nerve damage.5.Important vitamin for women of childbearing age and for older people.
1.Part of water-soluble B complex of vitamins. 2. It is essential in DNA synthesis, hematopoiesis & CNS integrity.3.Source for B12 is dietary foods like milk, yoghurt, eggs, meat, fortified cereals, bread. 4.Absorption depends on the HCl secreted by the stomach and occurs in intestines. 5. It is part of enterohepatic circulation, hence excreted in feces(approx. 0.1% per day)

Test interpretation

Higher than normal levels are in patients on Vitamin supplements or patients with COPD, CRF, Diabetes, Liver cell damage, Obesity, Polycythemia.

Decreased levels seen in

Inflammatory bowel disease, Pernicious anemia - genetic deficiency of intrinsic factor - necessary for Vit B12 absorption, Strict vegetarianslead to sub-clinical B12 deficiency- high among elderly patients, Malabsorption due to gastrectomy, smoking, pregnancy, multiple myeloma & hemodialysis, Alcohol & drugs like amino salicylic acid, anticonvulsants, cholestyramine, cimetidine, Hyperthyroidism (High levels of thyroid), Seen in mothers of children with (NTD) Neural tube defects- hence fortification and supplements are advised in expecting mothers

Recommendations-1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample. 2. Vit B12 and Folic acid evaluated together in macrocytic anemias to avoid methyl folate trap. Carmel's composite criteria for inadequate Vit B12 status: Serum vitamin B12 < 148 pmol/L, or 148–258 pmol/L and MMA > 0.30µmol/L, or tHcy > 13 nmol/L (females) and >15 nmol/L (males).

Associated Test-Holo-TC: Marker of vitamin B12 status -specificity and sensitivity better than serum vitamin B12, hence recommended in boderline and deficient cases for confirmation.

References-O-Leary F, Samman S. Vitamin B12 in health and disease. Nutrients. 2010 Mar 2(3):299-316.

FOLIC ACID, SERUM-Folates are compounds of pteroylglutamic acid (PGA) that function as coenzymes in metabolic reactions involving the transfer of single-carbon units from a donor to a recipient compound. Folate, with vitamin B12, is essential for DNA synthesis, which is required for normal red blood cell maturation. Human obtain folate from dietary sources including fruits, green and leafy vegetables, yeast, and organ meats. Folate is absorbed through the small intestine and stored in the liver.

Low folate intake, malabsorption as result of gastrointestinal diseases, pregnancy, and drugs such as phenytoin are causes of folate deficiency. Folate deficiency is also associated with chronic alcoholism. Folate and vitamin B12 deficiency impair DNA synthesis, causing macrocytic anemias. These anemias are characterized by abnormal maturation of red blood cell precursors in the bone marrow, the presence of megaloblasts, and decreased red blood cell survival.

Since both folate and vitamin B12 deficiency can cause macrocytic anemia, appropriate treatment depends on the differential diagnosis of the deficiency. Serum folate measurement provides an early index of folate status. However, folate is much more concentrated in red blood cells than in serum so the red blood cell folate measurement more closely reflects tissue stores. Red blood cell folate concentration is considered the most reliable indicator of folate status.

Methotrexate and Leucovorin interfere with folate measurement because these drugs cross-react with folate binding proteins.

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CONDITIONS OF LABORATORY TESTING & REPORTING

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
   i. Specimen received is insufficient or inappropriate
   ii. Specimen quality is unsatisfactory
   iii. Incorrect specimen type
   iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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